

<i>February 2014</i>	<i>2</i>
<i>August 2014</i>	<i>7</i>
<i>February 2015</i>	<i>12</i>
<i>August 2015</i>	<i>16</i>
<i>February 2016</i>	<i>20</i>
<i>August 2016</i>	<i>25</i>
<i>Feb 2017</i>	<i>28</i>
<i>August 2017</i>	<i>31</i>
<i>March 2018</i>	<i>35</i>
<i>August 2018</i>	<i>38</i>
<i>Feb 2019</i>	<i>42</i>
<i>Aug 2019</i>	<i>45</i>
<i>March 2020</i>	<i>48</i>
<i>October 2020</i>	<i>51</i>
<i>August 2021 Missing</i>	<i>54</i>
<i>February 2022</i>	<i>57</i>
<i>August 2022</i>	<i>61</i>
<i>2023 Feb</i>	<i>65</i>
<i>2023 - August</i>	<i>70</i>
<i>AUGUST 2024</i>	<i>73</i>
<i>MEQ March 2025</i>	<i>77</i>

Question 1 (18 marks)

You are a consult liaison psychiatrist. Jerry McIntosh is a 53-year-old secondary school teacher admitted under the medical team for new onset of neurological issues. He has had difficulty walking, memory difficulties, and he has not been sleeping, with irritability, and grandiose ideas. He believes he will cure the world of hunger through a new substitute food he has invented. Jerry is homosexual, and HIV positive for 30 years. The medical team are concerned that Jerry has become manic and are requesting your assessment.

1.1 Outline your approach to assessment of Jerry, including a cognitive assessment. (10 marks)

You diagnose Jerry with HIV dementia. Jerry's partner of 15 years, Bill is concerned about Jerry's recent decline. Bill comes to you and would like to know what is happening. Bill is worried about the impact of Jerry's decline on his work and their relationship.

1.2 Outline your response to Bill. (8 marks)

Question 2 (25 marks)

You are working as a consult liaison psychiatrist in the consultation liaison psychiatry service of a general hospital. You are contacted by your registrar on a medical ward with the message that the medical service medical service three days ago after an overdose involving multiple substances, not all of which are known. He is proving difficult to manage on the medical ward due to his wandering, agitation, and occasional aggressive outbursts without physical violence. He does not make much sense when he talks, and the medical team feel he is clearly psychotic and should be transferred to the inpatient psychiatry team.

2.1 Outline your approach to managing this situation. (6 marks)

Your registrar is able to tell you that the patient was brought in by the police after being found unconscious in his car. His blood alcohol on admission was 0.21% and he had a paracetamol level which was mildly elevated but not in the toxic range, although the exact time of ingestion was not known. Benzodiazepines and cannabinoids were also detected. On examination you find the patient to have a fluctuating level of consciousness. He is disoriented, highly distractible and over-active with repetitive, semi-purposeful movements. He is unable to give much coherent history and talks of seeing little creatures in his room. His affect appears cheerful, and he tries to be cooperative. He is somewhat flushed, and his chart indicates a mild pyrexia of 37.8 degrees.

2.2 What causes of the patient's condition should be considered and what other information (from any available source) would help support or exclude these potential causes? (10 marks)

Your registrar is able to get in touch with Theo's mother. She tells the registrar that Theo has had problems for some years with alcohol and marijuana. As far as she knows he does not use heroin or amphetamines. His pattern of alcohol use is mainly one of bingeing every few weeks and, when he can afford it, he uses marijuana daily. However, he lost his job a few months ago and since then he has been experimenting with readily obtainable psychoactive substances such as the pentadiene forte which his father is prescribed for back pain. After Theo's overdose his mother noted that supplies of his father's medication for back pain and Parkinson's disease are also missing - the bottles are "diazepam" and "benzhexol".

2.3 What is the most likely diagnosis? (2 marks)

2.4 What advice about immediate management will you give your registrar? (7 marks)

Question 3 (23 marks)

You are a child and adolescent psychiatrist. Ben is a 14-year-old boy referred to you by his GP for an assessment. Ben has not attended school for 6 weeks now. Ben attributes his school refusal to needing to sleep. Ben stays up late playing video games because he knows he won't be able to sleep. He finally sleeps around 3am. He wakes at 7am still feeling unrefreshed. Ben naps on and off from 5pm to 10pm. The GP has done some basic blood tests including FBE, UEC, LFT and TFT which were all normal. An examination revealed a healthy 14-year-old with normal body weight, respiratory examination and GI exam. Vital signs were all normal.

3.1 Outline your approach to assessment (10 marks)

You assess Ben. There are no psychological or neurovegetative features of depression. Ben has been having sleep issues for around one year, beginning following an episode of Glandular Fever. Other issues at home include fighting with his two younger siblings. His teachers report bullying at school for the past two months. Ben's school attendance and grades had been worsening over the past year.

3.2 List your differential diagnoses. (5 marks)

Ben's parents are very worried about his sleep difficulties and ask what can be done to help. Ben's mother begs you to give him something to help, as she can't sleep a wink either, with Ben's noisy computer game playing. She has heard that "Valium" may help.

3.3 Outline your response to Ben's parents, including advice for managing his insomnia. (8 marks)

Question 4 (26 marks)

You are a psychiatrist, specialising in veteran's mental health, and have been asked down to the emergency department by the eCAT clinician to assess Bill Anderson, a 67-year-old Vietnam veteran. He was brought in by police, following an altercation with the tram inspectors. Bill had reportedly become aggressive and threatening when asked for his ticket. Bill has been causing trouble in ED, shouting at co-patients that they are communists and demanding to leave. During the eCAT clinician's assessment, she observed that Bill appeared intoxicated, slurring

his words and smelling of alcohol. He was unkempt and appeared not to have showered in days. Bill was not known to mental health services and would not provide details for a next of kin, so limited information is available for him.

4.1 Outline the barriers to assessing Bill. (7 marks)

Three days on, Bill has been admitted to the veteran's ward of your hospital. It has been revealed that he lives with his 65-year-old wife Margaret. Margaret reports that Bill suffered immensely from his wartime experiences. Though he rarely speaks of them, she notices that he often mutters about communists in his sleep, waking in a cold sweat. Sometimes, when a door is shut too loudly, he will "jump out of his seat". He is often grumpy and moping about, but Margaret is used to this. He drinks around 6 stubbies of beer each day and has always done so. In the past few months, Bill has seemed out of sorts. He appears to have lost some weight and the other day; she heard him crying in his shed.

4.2 List your differential diagnoses. (5 marks)

4.3 Outline your short- and long-term management of Bill. (12 marks)

Question 5 (20 marks)

You are a part time psychiatrist working on an inpatient unit of a public hospital. Your registrar Jeremy is in his first year of training. You have been increasingly concerned about Jeremy. He appears unable to manage his workload effectively or prioritise. His clinical skills are also very poor, and he regularly "forgets" to do a risk assessment or mental state examination, or chart medications when admitting patients. You are aware it is his first rotation, and he must be anxious. You have been patient with him and provided extra supervision to help coach him through these tasks. Nonetheless, he does not appear to be improving.

He is often late to work, and you have noticed him falling asleep at his desk. He appears very distressed and frazzled when you or nursing staff provide feedback around things which have been missed.

5.1 Describe the reasons which may be contributing to Jeremy's difficulties. (8 marks)

5.2 Outline your approach to management of this situation. (12 marks)

Question 6 (28 marks)

You are the on-call psychiatrist and also work on the inpatient ward. You receive a call from your registrar about Sharon, a 35-year-old woman with chronic schizophrenia who is admitted under you. Sharon had been admitted earlier in the week for the purpose of a medication change. She had been managed with Haloperidol 6mg daily for many years in the community but had suffered akathisia in recent times which was causing great distress. Four days ago, you changed her to Quetiapine 1000mg with some effect. Sharon is now unwell with a fever. The registrar went to assess Shannon but found her asleep and thought better of waking her. The registrar was unsure how to proceed.

6.1 Outline your advice to the registrar around assessment. (10 marks)

Your registrar calls you back and sounds anxious. They have now assessed Shannon. They inform you that Sharon is asleep and difficult to rouse. On waking, Sharon appears confused. She is not responding to the registrar's questions but looks at them blankly. The nursing staff believe this is not normal for Sharon. On examination, she remains

febrile and hyper-reflexes. Her blood pressure is 180/90, heart rate 100, respiratory rate 20.

6.2 Outline your advice to the registrar around immediate management. (8 marks)

Sharon is now better, and you have ceased Quetiapine. Considering what happened, you are considering commencing her on Clozapine. Sharon's mother Mary is on the ward and would like to know what your plans are for her daughter.

6.3 Discuss your management advice with Mary and Sharon. (10 marks)

1. "HIV homosexual schoolteacher developing neuropsychiatric issues, psychosocial impact of it, cognitive domains needing testing.
2. Another on anticholinergic delirium, approach, treatment.
3. Another on 14-year-old with chronic fatigue, school refusal and sleep problems - differentials, management of insomnia, responding to benzo request.
4. Emergency assessment of someone intoxicated, veteran, probably PTSD. Questions about barriers to interview, differentials, short- and long-term management
5. Question on poorly performing intern - differentials, management
6. Chronic schizophrenia, on call, unwell with pyrexia after changeover to 1000mg quetiapine - approach to assessment, management, discussion of Clozapine afterwards

August 2014

Question 1 (34 marks)

You work as a consultant psychiatrist in a memory clinic. Maude is a 74-year-old woman who presented for evaluation of "short-term memory loss" at the behest of her family. Maude acknowledged memory difficulty but no more than her age-peers. Maude's daughter reports that difficulties in cognition and daily function were first noted about 3 years earlier, shortly after she was widowed. Her problems were attributed to grieving and depression, so she was treated with sertraline. Her mood and activity returned to normal, but she continued to struggle with managing the finances and shopping. This was attributed to her unfamiliarity with these activities, as her husband had done them exclusively over the last 5 years. More recently, she had discontinued her regular church attendance, had been noted to wear dirty clothing, and had been losing weight; however, her housekeeping remained immaculate. She had chronic atrial fibrillation and hypertension and had sustained a myocardial infarction 7 years earlier. Current medications included digoxin, warfarin, lisinopril, metoprolol, and raloxifene. She lived at home. Her older sister had 2 strokes and progressive memory difficulties in the 3 years prior to her death at 72 years of age. On cognitive examination, she was not oriented to date or month but could identify the day of the week. She could repeat 3 words but recalled none after 5 minutes of distraction. She could not recount any recent major news events. She could name a watch, pencil, and jacket, but could not name the lapel, sleeve, or cuff. She correctly drew a clock but could not set the hands to 8:20. General neurologic examination was unrevealing, apart from diminished large fiber sensation in the distal lower extremities, and a mildly unsteady gait. You think that Maude's cognitive deficits could be explained by a dementing process.

a. Please provide your differentials of the likely dementias and your rationale. (6 marks)

b. Please also provide all other possible non-dementia differentials that could explain Maude's current presentation. (4 marks)

You continue to review Maude in your clinic. Maude's daughter accompanied her to one appointment and expressed she is getting worried about Maude being alone at home. She had visited Maude once to find out that the door had been unlocked and Maude had left the kettle on the stove. She wants to know what she can do to help Maude.

c. Please outline your response to Maude's daughter and any practical advice that you can provide. (9 marks)

d. Please describe the different findings you would expect on CT, MRI and SPECT for the different types of dementias. (8 marks)

e. You are asked to give a presentation to the medical students on the neurobiology of Alzheimer's dementia. Please outline what you would include in your presentation. (8 marks)

f. During the talk, a medical student asks you if Alzheimer's dementia can be treated with medications. Please discuss the different pharmacotherapy used for Alzheimer's dementia. (7 marks)

Question 2 (25 marks)

You are a consultant psychiatrist in private practice. David, a 27-year-old man presents saying "Doctor I really think I need help. This can't go on anymore!" David states that he "always had a problem with handwashing", but lately this had been going out of control. He states that he had been more worried about germs on his hand and has to wash his hands multiple times a day. He also states that he frequently thinks he has forgotten to turn the tap off has to

check a few times to make sure he had done so. David states that lately when he was driving, he had been so worried about knocking down a pedestrian that he often had to circle the block a few times to check that there is no one lying on the road.

A. Please outline your short- and longer-term management for David. (10 marks)"

"You continue to treat David but 2 months later his father died suddenly from a cardiac arrest. David told you that since then his symptoms have worsened and he is now more worried about his cleanliness, and he is washing his hands and changing his clothes multiple times a day because of the risk of contamination.

B. Please discuss the relationship between David's obsessive-compulsive symptomatology and his father's death. (8 marks)

You feel that David is becoming more depressed after his father's death, and you diagnose him with major depression after 6 months. You recommend starting medications to help him, but David refused. You then decide to refer him to a community mental health clinic for follow up as he started to not turn up for appointments with you. You later find out that David had been making excuses not to engage with the community mental health team either.

C. Discuss why David may not have engaged with mental health services. (7 marks)

Question 3 (23 marks)

You work as a psychiatrist in an inpatient unit. Tom is a 24-year-old man who presents with a manic relapse of his bipolar disorder. He is managed on a CTO on risperidone depot and lithium. Tom had recently broken up with his girlfriend and because he has no further accommodation, he has decided he would move back with his parents on discharge. His father is an orthopaedic surgeon in your hospital and his mother is a GP. Tom discloses that prior to admission he was smoking "weed" quite frequently and tells you not to tell his parents because "they would freak". You have previously had a family meeting with Tom's parents and have noted that Tom's father does not like his son being labelled at "bipolar".

Please outline the potential complexities of Tom moving back to live with his parents on discharge (8 marks).

You hold another family meeting to discuss treatment and discharge. Tom's parents ask you if transcranial magnetic stimulation can be provided for Tom as they have read about this in a medical journal.

Please describe how you would respond to them (8 marks).

Tom's parents go home to do more research and comes back to tell you that they have now heard of social rhythms and interpersonal therapy was. They want to know if this would be suitable for Tom.

Please describe social rhythms and interpersonal therapy to Tom and explain if this would be suitable for him. (7 marks)

Question 4 (24 marks)

You are working as a Consultant Psychiatrist in an acute adult inpatient unit. You are informed by a nurse that whilst on leave out of the high dependency area, your 30-year-old patient Jane engaged in a sexual act. Jane is diagnosed with schizoaffective disorder and has had multiple psychotic relapse and admissions. She is now on Clozapine 600mg. She is currently an involuntary patient and has been nursed in HDU as she had absconded shortly after admission and had to be brought back by the police. Through reviewing the file, you are made aware that medical staff were not notified at the time of the event.

A. Please discuss the ethical issues raised by this scenario (6 marks)"

"Jane told you that she had sexual intercourse with a male patient in the courtyard in return for a cigarette. You later find out that she had a long history of IVDU and is HIV positive. She was offered antiretroviral therapy last year but has refused and so is not currently on any HIV treatment.

B. Please outline your approach to managing this situation. (9 marks)

2 weeks later, a blood test of Jane reveals an elevated bHCG and an ultrasound confirms the pregnancy. Jane is still in the high dependency area. She has ongoing psychotic symptoms, and she says she wants to keep the baby as she can hear angels voice telling her that the baby is a gift. Jane's mother storms to the ward and demands to know what can be done.

C. Please discuss your approach to managing this situation. (9 marks)

Question 5 (15 marks)

You are a consultant psychiatrist working in a public service. You are on call one weekend and the registrar calls you for advice. Karly is a 20-year-old female who presented to ED after taking an overdose of 20 tablets of paracetamol. Her paracetamol levels were not high enough to need a NAC infusion and she is now medically cleared by ED. Karly had taken the overdose in the context of stress over going to a family reunion tomorrow. She claims that her grandfather had sexually abused her when she was a child and that she had not seen him in over 10 years. She tells the registrar that if she sees him she will want to "get him back for what he did to me". Your registrar tells you that Karly is quite agitated and teary but "has promised" the registrar she would not do anything else silly. Your registrar wants to know if he can send Karly home.

A. Please outline your response to the registrar. (8 marks)

2 months later, you receive a call from a psychologist Jade. Jade has been working with Karly to deal with her history of sexual assault. Jade tells you that she believes that Karly still has a lot of repressed memories surrounding the abuse and wants to know if you can help with the situation by providing advice or perhaps medication to "calm her down" as Karly is often quite distressed after sessions with Jade.

B. Please discuss the ethical issues arising from this situation. (7 marks)

Question 6 (19 marks)

You are a psychiatrist in a CL team. Richard is a 69-year-old man who presents with a 1-year history of functional decline. His wife reports that his symptoms developed insidiously but have mildly progressed. The medical team told you that he is not eating much and "looking flat" so he must be depressed. However, on your assessment you suspect that he might have Parkinson's disease.

A. Please outline the symptoms and signs that Richard could present with if he had Parkinson's Disease. (6 marks)

Richard was diagnosed with Parkinson's Disease and started on treatment. A month later he was readmitted again, and the neurology team reports that Richard is expressing suicidal ideations, and his wife is saying that Richard is

sitting on the sofa all day doing nothing. He has at times looked at her suspiciously and had recently asked her what she had put in his food.

B. Please describe your assessment of Richard. (10 marks)

C. Please outline the significance of dopaminergic agents in gambling disorder. (3 marks)"

Dementia question with multiple components

- Differentials for some presenting with a cognitive deficits,
- 2 separate questions, 1 on dementia differentials, the other on all other possible differentials
- Practical advice to the husband of the patient
- Findings you would expect on CT, MRI and SPECT for dementias
- Presentation to medical students on Alzheimer's disease neurobiology
- Presentation on different pharmacotherapy used for Alzheimer's disease

1. Question about a man with severe OCD, psychological treatment, pharmacological treatment, how his condition could worsen when his father has a sudden cardiac arrest and dies, why he may not have engaged with a mental health service.
2. Bipolar disorder in a man on a CTO on risperidone depot and lithium who moves back to live with his parents who are both doctors, past history THC. Had to outline the potential complexities of this. Parents request some weird electrical stimulus treatment, must explain how you would respond. How to handle their queries in future. Had to explain what social rhythms and interpersonal therapy was.
3. Question with multiple ethical issues about a woman on clozapine in SECU-type place with HIV who has a sexual relationship with another inpatient on the ward and gets pregnant.
4. Question about a patient presenting to ED with a non-toxic paracetamol overdose in the context of stress over going to a family reunion with a grandparent who sexually abused her. Ethical issue over a therapist who is working with her down the track to uncover repressed memories.
5. Parkinson's disease – signs and symptoms Risk assessment / clinical assessment in someone with suicidal ideation and Parkinson's Disease 3-mark question of significance of dopaminergic agents in gambling disorder

Question 1: Child and Adolescent Psychiatry

Did not specify if you were a registrar or consultant (unlike the rest of the MEQ's where you were the consultant). I think this is a repeat question. I think I've seen this somewhere amongst the past papers available on the RANZCP website.

You work in a Child and Adolescent Outpatient Clinic, and a paediatrician has sent you a referral. 10-year-old boy attends with his mother. Lives at home with parents and siblings. Paediatrician has diagnosed him with ADHD and Tourette's, prescribed him methylphenidate and clonidine with improvement of behaviours at home. However now poor school performance.

1.1 What are some of the possible reasons for his poor school performance?

Mother reports she has read in a magazine article that stimulants cause growth retardation and "drug addicts".

1.2 How do you respond to mother's concern about medication?

You continue to prescribe him his medication and his school performance improves. However, his mother is concerned about ongoing tics.

1.3 Outline your management of his Tourette's.

Question 2: Adult Psychiatry

You are the consultant of an inpatient psychiatric ward. Your patient is a 51-year-old man with psychotic depression. He appears to be improving with medication, so his nursing observations are downgraded. When you arrive to work early Monday morning, you are informed that your patient suicided over the weekend. He was found hanging in his bedroom on the ward.

2.1 What is your immediate response to this situation?

Your patient's ex-wife calls you. She has been informed by the police of the death. She asks for advice on what to tell their 10-year-old son. He last saw his father 2 months ago. She is not sure whether to tell him about the suicide.

2.2 What is your response?

During the week you notice that the nursing staff are reluctant to downgrade nursing observations or to let inpatients go on leave. They request more nursing staff as they report they are unable to provide adequate nursing care.

2.3 What are the possible reasons why the nursing staff are behaving like this?

Question 3: Indigenous and Adult Psychiatry

You are the consultant of a community mental health clinic. Your patient is a 49-year-old Indigenous woman who has been case managed and treated for symptoms of PTSD for the last 6 months following a sexual assault 2 months.

On her way to her appointment today, she saw her assailant, who is a distant relative, on the bus and became very distressed. Her case manager has just approached you with concerns about the level of her distress and risk of self-harm.

3.1 Before assessing your patient, what information do you need from the case manager?

You see your patient with her case manager. She reports after speaking to you she feels much better. She denies any self-harm ideation and does not want to be admitted to hospital. She would like to go home and would like something to “make me feel better and help me sleep tonight”.

3.2 What is your immediate management plan?

A week later the case manager approaches you and informs you that your patient is feeling much better after speaking about her symptoms. Your patient would like to disclose the sexual assault to her adult daughter. However, she is concerned that her adult son, and other members of the community will find out.

3.3 What is your response to the case manager?

Question 4: Consultation Liaison and Cultural Psychiatry

You are the consultant of the consultation-liaison service. You ask your registrar to review a referral from ED of a Filipino mother of a 4-year-old son. Her husband works offshore frequently. She attended ED for what staff have diagnosed as a “pseudo-seizure”.

She does not have many friends. She reports her son sleeps poorly and she is struggling to look after him.

4.1 What salient features on history, examination and formulation, do you ask your registrar to focus on?

The patient is very anxious about returning home. Your registrar believes she needs to be admitted. The social worker is unable to find alternative care arrangements for her son. Her husband has called and angrily said that “she is attention-seeking” and should return home to look after their son. ED are demanding that she be discharged as they need the bed. Your registrar anxiously asks for your advice.

4.2 What is your response to your registrar?

Question 5: Adult Psychiatry

You are the consultant of a community mental health clinic. Your patient is a 39-year-old woman with a 10 year history of chronic schizophrenia. Despite good adherence with medication, she continues to experience distressing auditory hallucinations. She is prescribed oral risperidone 6mg nocte. She complains of extra-pyramidal side effects. She requests a change of medication.

5.1 What features on history and examination do you need to inform your decision about medication?

On examination she has tremor and stiffness.

5.2 How do you manage her extra-pyramidal symptoms? (3 marks)

You change her to clozapine. After 3 weeks, she is on 200mg a day of clozapine and attends for her routine clozapine review. She reports experiencing a one-off episode of hot and cold overnight. Her WCC is normal, her cardiac enzymes (CK and trop) are normal. Her ECG shows an increased QTc from 430 previously to 503. Her physical observations are normal.

5.3 What is your management plan? (10 marks)

You decide not to admit her and see her at clinic in 3 days. On review she reports nausea, vomiting, fever, chills, dizziness since last night. On examination she has a fever and is tachycardic.

5.4 What are your differential diagnoses (explain your rationale, and how you would confirm/exclude these)? (6 marks)

Question 1: Child Psychiatry

6-year-old boy with encopresis, mother is on single parent support. Referred by his paediatrician who has been treating him with laxatives and star chart for six months to no effect. He has been reviewed by a surgeon to exclude medical causes

1.1 Outline your assessment (6 marks)

The boy's grades are normal, and he has friends, though the kids sometimes make fun of him for being smelly.

1.2 Describe your management

His mother informs you that the father is out of the picture. She feels her son is "violent" like his father. She is also getting tired of "rewarding him, when he doesn't deserve it".

1.3 Describe your management

Question 2: Adult Psychiatry

22-year-old man is referred to you by his GP. He believes he has ADHD. His younger brother (11 years old) has been diagnosed and is receiving treatment for this (methylphenidate).

2.1 Describe how you would confirm the diagnosis?

The man admits to using his younger brothers' medications sometimes which he feels are helpful. He describes some minor driving accidents. He gives a history of onset of symptoms in early primary school.

2.2 Outline your management

You are asked to give a talk to GPs on ADHD.

2.3 Discuss the key issues you would consider in your talk (14 marks)

Question 3: Aboriginal psychiatry

You are a rural psychiatrist. Your patient is a 52-year-old Indigenous man with chronic schizophrenia managed with Flupentixol depot 20mg IMI 2 weekly. The man lives with his son, daughter in law and 2 grandkids (3 and 5 years old). In the past when unwell, he has responded to an increased dose in his medications. He has a past history of

verbal aggression and paranoia toward the neighbours. It seems he is deteriorating in his mental state at present. His son thinks this is because his uncle died recently.

3.1 Outline the risks inherent in this scenario.

The son really wants his father to be managed at home. He believes he can convince his father to take some oral medications (olanzapine). You agree to the plan.

The cultural liaison worker informs you that he believes this is a culturally normal grief reaction, rather than a relapse of schizophrenia. He feels that the patient should not be managed with medication, that he should be managed with traditional healing instead.

3.2 Outline your response

The patient appears to have become paranoid about the neighbours again. He has also been looking suspiciously at his grandchildren. His daughter in law no longer feels safe with him in the house and wants him out. The son Robert tells you about this and states he feels torn between supporting his wife and his father.

3.3 Describe your advice

Question 4: Perinatal psychiatry

You are a private psychiatrist. Your patient is a 35-year-old woman with Bipolar disorder who lives at home with her husband, their 2 children (18 months and 3 years old). The patient has a history of postpartum psychosis. She is currently managed with Sodium Valproate 1600mg and Olanzapine 20mg. She has gained a lot of weight on this regime (over 10kg) and is really upset about this. She would like to stop taking her medications.

4.1 Outline your response to her request

She agrees to continue medication but is still concerned about weight

4.2 Describe your management plan.

She agrees to continue her medications. She has successfully lost 4kg and is happy with this. She would like to have the third child she has always planned for.

4.3 Outline your advice

Question 5: Consultation Liaison Psychiatry

You are a CL psychiatrist. The spinal team refer you a patient following a motorbike injury. The patient is complaining pain and there are behavioural issues which are disruptive on the ward. The patient has a history of cannabis use and is a smoker and demands to use these in hospital or he will discharge himself. The staff are very stressed. Your registrar thinks he has an antisocial personality disorder.

5.1 Describe to your registrar the differential diagnoses which must be excluded

5.2 Outline your advice your registrar around how to manage cannabis and nicotine withdrawal.

The patient is demanding opioids.

5.3 Describe your advice your registrar on the key issues in managing this patient's pain.

Question 6: Adult psychiatry

You are a community psychiatrist. You are referred a patient, Duncan, following a paracetamol overdose. The patient is a middle-aged accountant who lives at home with his wife and 2 school aged children. The patient took the overdose following an argument with his wife in which she caught him looking at pornography.

6.1 Outline the reasons

The pornography was adult, and not homosexual. You are now providing marital counselling to the patient and his wife.

6.2 Describe the key issues which you will explore in your therapy.

6.3 Outline the barriers to engaging Duncan in therapy.

Duncan has been caught looking at pornography at work and will be reprimanded for this. He asks you to write a report.

6.4 Discuss your key considerations.

Question 1: Adult psychiatry (24 marks)

You are the consultant treating Paul, aged 51, who was an inpatient for ten days in the acute psychiatric unit in which you work. He was admitted with a diagnosis of major depressive disorder with psychotic features. He was commenced on fluoxetine 20 mg daily shortly before admission. Risperidone 3 mg daily was added on admission. Paul's mental state began to improve, and the level of nursing observation was reduced. When you arrive at work early on Monday morning, you are told that Paul had hanged himself in a ward bathroom during the weekend and that attempts to resuscitate him had been unsuccessful.

1.1 Discuss your approach to this situation. (8 marks)

You receive a phone-call from Paul's ex-wife, Helen, who has been contacted by the police as next-of-kin and informed of his death. Helen wants to know what she should say to her twelve-year-old son, Tim, as he does not yet know that his father has died. Helen wonders if she should avoid telling Tim that the cause of death was suicide. Tim last saw his father two months ago when he stayed with him for the school holidays.

1.2 Describe the information and advice you would give to Helen to help with Tim. (8 marks)

Four days later, you notice that the nursing staff are keeping all of the patients on high levels of observation and are reluctant to let any patients have leave. The nurses are also complaining that there are not enough staff to provide adequate patient care.

1.3 Discuss the issues that may be contributing to these observations. (8 marks)

Question 2: Perinatal psychiatry (24 marks)

You are a private psychiatrist. Your patient is a 35-year-old woman with Bipolar disorder who lives at home with her husband, their 2 children (18 months and 3 years old). The patient has a history of postpartum psychosis. She is currently managed with Sodium Valproate 1600mg and Olanzapine 20mg. She tells you she has tried other medications in the past but these were ineffective. She has gained a lot of weight on this regime (over 10kg) and is upset about this. She tells you she would like to stop taking her medications.

2.1 Outline your response to her request (8 marks)

She agrees to continue medication but is still concerned about weight.

2.2 Outline your management plan regarding weight management. (6 marks)

She has successfully lost 4kg and is happy with this. She would like to have the third child she has always planned for.

2.3 Outline your management plan regarding medications. (6 marks)

She is considering taking Lithium.

2.4 Outline your advice about Lithium. (4 marks)

Question 3: Adult psychiatry (24 marks)

You are the psychiatrist at a community mental health clinic. John is a 38 year old man who used to work as an electrician, but is now unable due to his OCD. He lives with his elderly parents. He has a long history of OCD characterised by obsessions around hygiene and checking rituals of taps and doorknobs. His parents do most things for him including shopping, cooking and cleaning. He has had a long history of erratic involvement with mental health services.

3.1 Outline the possible reasons for his poor engagement with mental health services. (6 marks)

You conduct a home visit and meet with John. He denies problematic substance use or symptoms of depression. He is agreeable to commence treatment as long as it doesn't involve admission.

3.2 Outline your psychological management of John. (8 marks)

3.3 Outline your pharmacological management of John. (4 marks)

The case manager informs you that John's father died.

3.4 Outline the possible consequences for John. (6 marks)

Question 4: Aboriginal psychiatry (22 marks)

You are the psychiatrist at a community mental health clinic. Jean is a 45-year-old woman of indigenous background. She has a history of sexual abuse and now suffers from Post Traumatic Stress Disorder characterised by nightmares, hypervigilance and agitation. She has recently seen her assailant, who is an older relative at a family function. This has triggered her symptoms and caused her immense distress. You are contacted about this by Jean's case manager who is concerned about Jean's risk of self-harm.

4.1 Outline the information you would like to seek from the case manager prior to reviewing Jean. (8 marks)

Jean states she feels better now, she denies intent to harm herself and tells you "I just want to go home so I can get some sleep".

4.2 Outline your advice to the case manager on how to manage Jean. (8 marks)

Jean has told her case manager that she has had great relief just from talking. She would like to inform her daughter of her history of abuse, however she is fearful that her son or others in the community may find out.

4.3 Outline your advice to the case manager on how to manage Jean. (6 marks)

Question 5: Consultation Liaison Psychiatry (18 marks)

You are a CL psychiatrist. You are referred Bill by the endocrinology team. Bill is a 45-year-old married man, previously working as an electrician, who has a history of pituitary carcinoma. His tumour was successfully removed by the endocrinology team. However, since that time, Bill has developed mood lability, social withdrawal and low self-esteem. He initially returned to work as an electrician. However, he retired from work after six months as he had lost his confidence. He also suffers from chronic headaches and insomnia.

Bill has sleep apnoea and sleeps with a CPAP machine. He is managed with thyroxine and corticosteroids.

5.1 Outline the possible reasons for Bill's mood lability (6 marks)

You meet with Bill and his wife. Bill's wife has taken over cleaning, shopping and management of the bills since Bill's surgery. Bill feels angry and frustrated about this. According to the Occupational Therapist, Bill is capable of doing these things himself.

5.2 Outline your management of this situation. (8 marks)

The endocrinology team tell you that Bill's tumour has returned, although it is treatable. Bill is becoming increasingly irritable and labile in his mood.

5.3 Outline the possible causes for Bill's mental state deterioration. (4 marks)

Question 6: Consultation Liaison psychiatry (28 marks)

You are the consultant of the consultation-liaison service. Your registrar comes to you for advice about a new referral from ED. Andora is a woman of Filipino background with a 4 year old son, Ben. Her husband works off-shore on an oil rig and is often away. She attended ED for what staff have diagnosed as a "pseudo-seizure". Andora does not have many friends. She reports Ben sleeps poorly and she is struggling to look after *him*.

6.1 Outline your advice to your registrar on what to focus on the history. (12 marks)

6.2 Outline your advice to your registrar on what to focus on in examination. (6 marks)

6.3 Outline your advice to your registrar about the key structural features of a formulation. (4 marks)

The patient is very anxious about returning home. Your registrar believes she needs to be admitted. The social worker is unable to find alternative care arrangements for her son. Her husband has called and angrily said that “she is attention-seeking” and should return home to look after their son. ED are demanding that she be discharged as they need the bed. Your registrar anxiously asks for your advice.

6.4 Outline your response to your registrar. (6 marks)

Question 1: Neuropsychiatry

Maria is a woman with low IQ, who presents with persecutory ideation post seizure (repeat question from a past exam).

1.1 Outline your assessment

1.2 What is your differential diagnosis?

1.3 Outline your management

Question 2: Aged psychiatry

Behavioural and psychological symptoms of dementia including talk to carers and a 'debate' with the carers regarding management.

2.1 What are the principles of management of BPSD

2.2 Describe how you would give a talk to carers

Question 3: Perinatal psychiatry

A woman who is 8-weeks pregnant woman is prescribed sodium valproate, diazepam and paroxetine.

3.1 What recommendations would you make regarding her pharmacological management?

3.2 Describe IPSRT and how it is relevant in this case

Question 4) Adult psychiatry

A migrant middle-aged man presents with low mood and a headache.

4.1 What is your differential diagnosis?

4.2 Outline your assessment of this man.

4.3 Outline your management of this man.

Question 5: Adult psychiatry

Your patient is a young man who has schizophrenia. The young man's father is a doctor and has been prescribing your patient his medication.

5.1 What are the ethical and legal issues

5.2 Describe the evidence base for involuntary treatment orders

Question 6: Adult psychiatry

A woman has a background of childhood sexual abuse. She is admitted to hospital after her daughter found her in the midst of a suicidal act.

6.1 Outline the immediate and longer-term management

6.2 What are the interventions for her family

Question 1: Child psychiatry

12-year-old boy with ADHD and tics starting compulsive hand washing

1.1 What is your approach to assessment

He is diagnosed with OCD.

1.2 What is your management

Question 2: Addiction psychiatry

40-year-old married lady with two teenage children, she is experiencing prescription codeine dependence.

2.1 What key features would you explore in the history?

2.2 What is the management of codeine dependence, what are the pros and cons of this?

The daughter becomes clingy and oppositional.

2.3 How would you manage the daughter?

Question 3: Consultation liaison psychiatry

You are a CL Psychiatrist, asked to see 4-year old girl with abdominal mass, turns out to be metastatic cancer. Mother part-time accountant. Father shouting, crying, possibly responding to auditory hallucinations on the ward, and refusing any blood tests or management for daughter.

3.1 What do you want to know from the treating team

3.2 Why was father acting this way?

3.3 What are the professional or ethical issues

Question 4: Addiction psychiatry

50-year-old single mother on DSP, has a 15-year old. ED registrar calls saying she has alcoholic pancreatitis and hallucinations

4.1 Further information you want from the referring ED doctor

4.2 List and justify differential diagnosis

Her BAL is 0.04, no hallucinations but has blurred vision on lateral gaze / nystagmus.

4.3 What is your differential diagnosis and immediate medical management?

Question 5: Adult psychiatry

36-year-old female medical officer. Depressed for 2 weeks. 2 previous depressive episodes, treated with sertraline for 9 months each time.

5.1 Further information before management

She became hypomanic after one week of treatment.

5.2 What is your management plan now?

Clinical director has requested to speak to you about the patient as her work is suffering.

5.3 What are your ethical, professional and legal issues?

Question 6: Aged psychiatry

72-year-old lady acting bizarrely, no sleep. On levodopa, lithium, fluoxetine, atenolol. Presents with cognitive impairment, global MMSE 22/30, bad clock face, acting bizarrely.

6.1 Outline your differential diagnosis for bizarre behaviour

6.2 Principles for prescribing in the elderly

6.3 Son has requested you assess for testamentary capacity, how is this done?

6.4 What is the information she needs to appoint a EPOA for her property

Question 1: Aboriginal psychiatry (31 marks)

You are consultant psychiatrist visiting a small town of 400 people a few days each month. You see a 56yo Aboriginal woman from Wiradjiri country. Previously assessed for anxiety but did not engage and Aboriginal MH nurse felt she was ok so she was discharged to care of GP. Presents now with complaints of increased anxiety and fear of leaving the house in the context of the recent release from prison of the man who sexually assaulted her 25 years ago. He has returned to the town and she fears seeing him. Extremely distressed, but not suicidal, psychotic or depressed.

1.1 What are the cultural considerations for your assessment? (6 marks)

You diagnose her with PTSD.

1.2 What factors may affect her engagement in treatment? (10 marks)

She asks you for a script for diazepam and you discover she has been taking 40mg daily for several years. Her GP neglected to mention this in the referral letter.

1.3 How would you respond to her request? (8 marks)

1.4 What implications does her benzo use have on treatment? (7 marks)

Question 2: Aged psychiatry

Man in his 80s in a high-level care nursing home with severe Alzheimer's dementia. Has episodes of verbal aggression and at times hits the staff. Some of the nurses are relatively untrained and believe he is behaving this way deliberately.

2.1 You ask the nurses to collect some information about his behaviours before your visit in a few weeks. What information would you like them to collect? (10 marks)

2.2 When you next visit in 2 weeks you discover that they have not collected any information. What factors may contribute to them not collecting this information? (10 marks)

2.3 The manager asks you to conduct a case discussion with the staff to engage them in behavioural interventions. What would you cover in this case discussion? (11 marks)

Question 3: Adult psychiatry (30 marks)

Man in his 30s who is married and has a child. Corporate account, well-respected at work, and is expected to become a partner soon. Volunteers on weekends with disadvantaged youths. Presents on request of his wife after she discovered him looking at adult pornography online.

3.1 What are the possible reasons for his use of pornography? (8 marks)

3.2 He is reluctant to talk about it with you – why might this be?

3.3 You decide they need couples therapy. What are the key issues you would explore in couples therapy? (10 marks)

After a few sessions he reports that he spends large amounts of time watching pornography and gets pleasure from it. He reports he was found using a work computer to watch pornography and is facing disciplinary action at work. He asks you to write a report for his employer.

3.4 How would you respond to this request? (12 marks)

Question 4: Aged psychiatry (31 marks)

Man in his 70s is taken to ED after a serious suicide attempt by hanging. No evidence of depression, psychosis or psychiatric morbidity. He says he has lived a full life and would like to die. He is reviewed by the ED registrar who rings you for advice on whether to detain him.

4.1? something about your approach to the situation in regard to detention in hospital (11 marks)

The man gets admitted for further observation and assessment. During the admission, his son arrives from overseas and sees you in a face-to-face meeting. He says that his father will kill himself if discharged.

4.2 What would you discuss in this meeting with the son? (10 marks)

A week after discharge, the man completes suicide. You discover that your registrar did not handover to the community team for follow-up, as had been planned.

4.3 What would you do? (10 marks)

Question 5: Adult psychiatry (17 marks)

Woman in her 30s with bipolar disorder. Married and has 2 preschool-aged children. Has had multiple previous admissions for mania and an episode of severe post-partum psychosis. Currently treated with valproate and olanzapine 20mg to good effect. Comes to see you in the community complaining of weight gain. Has put on 20kg since being on these meds and has tried to lose weight through diet modification but has struggled to do so.

5.1 She asks if she can stop her medications because of her weight gain. What is your response? (7 marks)

She manages to lose 6kg through use of new strategies. She now reports that she and her husband want to have the 3rd child they have always wanted and asks for your advice about her medication for pregnancy planning.

5.2 What is your response? (10 marks)

Question 1: Aboriginal psychiatry

Repeated question on older Indigenous man living with his family with thoughts to burn the house down.

Question 2: Child psychiatry

Referred by GP to community psychiatry centre. Patient is Britneigh, a 14-year old girl with chronic fatigue and various somatic symptoms. She has been home schooled for the past two years. She is isolated from her friends. She stayed up until 3am watching television and using social media. The GP has excluded physical causes for her chronic fatigue and somatic symptoms.

2.1 What is your approach to assessment?

2.2 What are the possible differentials?

Britneigh and her mother request hypnotics to help Britneigh sleep. She has previously used hypnotics to help her sleep, however these have had little effect.

2.3 What is your approach to addressing this request?

Question 3: Consultation liaison psychiatry

You are a consultation liaison psychiatrist who has been asked to see a 45-year old lady with a history of depression and hypomania. She has been hospitalized with psychotic depression, is not communicating and not eating or drinking for the last 48 hours.

3.1 What are the ethical and professional principles you will discuss with your registrar?

3.2 The patient's family say they are opposed to ECT. How would you approach a discussion about ECT with her family?

3.3 What are the indications for ECT?

3.4 What is your clinical approach to the management of a patient about to receive ECT?

Question 4: Adult psychiatry

You are a psychiatrist who is asked to see a 40-year-old man who is a used car salesman with no psychiatric history. He presents with suicidal intent after being accused of committing a sexual assault at his workplace.

4.1 What should you ask the registrar to assess?

4.2 What factors make you think he needs an inpatient admission?

4.3 The patient's solicitor requests you provide a psychiatric report. What advice would you give to the solicitor?

Question 5: Aged psychiatry

A 56-year-old man presents with a unilateral tremor, having noted some increasing difficulties with his writing and typing, which has made him so depressed that he has developed suicidal ideation. His general practitioner has diagnosed Parkinson Disease. He refers to you for assessment of this man's suicidal ideation.

5.1 What are the signs and symptoms of Parkinson disease? List and justify

5.2 What are the signs and symptoms of suicidal depression, work difficulty with typing. Outline the relevant psychiatric issues.

The patient is commenced on cabergoline therapy eight months ago. For the past five months, he has been experiencing increased obsessive symptoms, including fixing bicycles and pulling apart computers. His wife is concerned.

5.3 Explain the possible reasons for this behaviour.

August 2018

MEQ 1 (32 marks)

35-year-old single woman, living alone, with a history of schizophrenia. Managed by community mental health team. Taking risperidone 6mg. Despite this, she still experiences frequent auditory hallucinations. She has tried olanzapine in the past but suffered significant weight gain. She comes to see you for advice about her treatment.

1.1 What information do you need to know for your assessment? (11 marks)

She reports significant EPSEs – tremor and stiffness. Interfering with her everyday activity.

1.2 What are your management options? (6 marks)

She decides to try clozapine, which is titrated to 200mg daily. She reports feeling hot and cold. Her initial investigations (troponin, CK, CRP) are normal. ECG shows QTc 470.

1.3 What is your management plan? (6 marks)

She stays on the clozapine at the same dose. The next day, she becomes seriously unwell (cardiac) and is admitted to hospital, requiring ICU. Her mother is angry and accuses you of negligence.

1.4 What is your response? (9 marks)

MEQ 2 (repeat from Feb 2017) 24 marks

Kelly is a 4-year-old girl, admitted to hospital with an intrathoracic mass. It is probably malignant. She lives with both parents and her two older siblings, age (7 and 11 years old). Her mother works as an accountant. Her father (Wayne) has been yelling at staff and crying on the ward. The staff think he might be hearing voices. His responses have been related to his daughter's situation, and he has been refusing any blood tests for his daughter.

2.1 What information do you want to know from the treating team? (8 marks)

2.2 What are the possible explanations for Wayne's behaviour? (6 marks)

2.3 What are / debate the professional and ethical issues? (10 marks)

MEQ 3 28 marks

8-year-old girl living with foster parents, with a history of coming from a violent home. She reports hearing voices that are scary, transient, and interfere with her from sleeping. She lives in a large regional centre. Her GP refers her for assessment.

3.1 What would you want to know in her assessment? (10 marks)

3.2 What are the possible explanations for her problems? (6 marks)

3.3 What are the potential problems with diagnosing a primary psychotic disorder in this patient? (4 marks)

You have ruled out a diagnosis of a primary psychotic illness. You believe her voices are likely related to her past trauma.

3.4 What is your initial management plan? (8 marks)

MEQ 4 (August 2017 repeat) 28 marks

Amanda is a 34-year-old, married mother of 2 pre-school aged children (18 months, 24 years). She has a diagnosis of bipolar affective disorder with multiple previous admissions for mania. She had an episode of severe psychotic depression after the birth of her second child. She is currently taking valproate 1600mg and olanzapine 20mg. She has put on 20kg on this regime. She comes to see you asking to cease her medications. *The question may also have made a comment about her feeling that she has tried other mood stabilisers and that they had not been effective for her.*

4.1 How would you address her concerns (8 marks)

She decides to stay on the same doses of her medications. She comes to see you for advice about weight loss.

4.2 What is your management plan? (6 marks)

She manages to lose 6kg through use of new strategies. She now reports that she and her husband want to have the 3rd child they have always wanted and asks for your advice about her medication during pregnancy.

4.3 What is your response? (10 marks)

She decides to take lithium. *This was recalled by some as being unusual given her previous comment about mood stabilisers not being effective in the past*

4.4 What do you discuss with her? (4 marks)

MEQ 5 28 marks

A 14-year-old girl, living with both parents, presents to community mental health service due to 15kg weight loss in short period of time, with a BMI of 15. She has been restricting her food intake, only eating leafy greens. She is exercising excessively. She is afraid of gaining weight.

5.1 What are the key aims of the initial assessment? (8 marks)

5.2 What physical examination and investigation findings would indicate a need for admission? (6 marks)

She is admitted to the paediatric inpatient ward.

5.3 How do you reduce the risk of refeeding syndrome? (6 marks)

Her condition is stabilised, and she is discharged home. She reports that her father is never home, and her mother is controlling. There was disagreement in recall, with another registrar recalling that her father was controlling, and mother had her own issues with food

5.4 What would you tell her parents about treatment options? (8 marks)

MEQ1

You are a junior consultant in a community clinic and a GP refers you a 71y male presenting with likely major depression somatic complaints. Widower, lives alone, has early morning waking, poor sleep, low mood, hopelessness, constipation and concerns over being physically unwell. GP ruled out physical illness. Not eating anymore.

- **He refuses to go to hospital. List factors required to consider management (10 marks)**
- **Pharmaceutical management of this patient (10 marks)**
- **nursing assessment (10 marks) when he gets to the ward**

MEQ2

You are a junior consultant in a community clinic and are referred a lady with postnatal depression after the birth of her two children wants to have another baby. On venlaxine 225mg. Husband ambivalent about baby.

- **Wants to stop her antidepressant what issues will you discuss with her (10 marks)**
- **Husband doesn't want baby and comes to meeting. How would you facilitate discussion between them (6 marks)**
- **She's 8 weeks pregnant and she stopped meds (GP confirmed pregnancy). She is now irritable, low mood, early morning wakefulness. How would you manage her now ? (8 - 10 marks)**

MEQ3

You are a consultant in community overdose clinic and referred a patient who took an amitriptyline overdose. She something (doesn't mention she's an immigrant initially). 42-year-old married female has one child who's 3 years of age. Stopped working as a university tutor and not looking after her child. Husband working away a lot. Presenting with abdominal pain, muscle pain worse on exertion, poor sleep, low mood.

- **Differential diagnoses with justification (8 – 10 marks)**
- **Husband comes and says he does not want to see psychiatrist as their community will stigmatise them. What are the causes of stigma in this case? (6 marks)**
- **Does not attend follow up. List and justify the issues around non-engagement that you would discuss with your supervisor (10 marks)**

MEQ4

You are a consultant psychiatrist working for a CL service

Maria is a 25-year-old female with epilepsy (partial and complete seizures) that is treatment resistant. Lives with parents in a country town. Intellectual impairment with measured IQ of 65. She occasionally manages basic work? factory several hours a week. Long history of epilepsy.

Presents in status epilepticus and goes to ICU for three days. She then accuses nurses of spying on her, talking about her, poisoning her food.

- What information would you expect the psychiatry registrar to collect prior to seeing Maria (12 marks)
- List and justify diagnostic possibilities (6 marks)
- The neurologist says she continues to be at risk of seizures and needs to stay on the medical ward. Nurses state they want her moved as her behaviours are causing distress to other patients. How would you manage this situation? (10 marks)

MEQ 5

Toby is 25-year-old male who has come to ED late at night requesting a repeat diazepam scripts (with 7 repeats). He has no previous history with services.

- What information do you expect the registrar to have obtained during their assessment? (10 marks)
- He becomes aggressive and threatens to self-harm if he does not get his script of diazepam, saying he's being treated by a junkie. How would you manage this situation? (7 marks)
- He doesn't attend his outpatient appointment, and the registrar is informed that the hospital has received a complaint from his poor care of ED and the hospital want a response from this. How would you advise your registrar to manage this situation? (7 marks)

Aug 2019

Question 1

28yo man with schizophrenia. Working as a radio announcer. One admission at age 22 for 3 months. At the time, lived with a flatmate who reported low mood and vomiting. Has been on paliperidone 150mg depot (for 4 years?). Is on a CTO. Case manager asks you to review the patient before the MHT hearing.

1.1 Outline the information you want to obtain from the case manager about the history.

1.2 ?

1.3. The MHT members ask you about the risks of taking paliperidone. What do you tell them?

1.4. Outline what you would consider when deciding about their treatment order

Question 2

You are the supervisor of a stage 2 psych trainee. You meet to discuss supervision.

2.1. What are the principles of supervision?

2.2 What are the keys to giving good feedback?

Question 3

34yo male, ex-electrician, with OCD. He cannot leave home. Multiple obsessive thoughts and checking behaviours. Fears that he needs to keep checking power points and has dreams that his house burns down and killed his parents due to faulty electrics. Lives with elderly parents who shop for him. Engaged with mental health services previously but poorly. Tried multiple medications but discontinued due to intolerable side-effects.

3.1. List and explain possible reasons for poor engagement.

3.2. List and explain psychological treatment options.

3.3. List and explain pharmacotherapy options.

Question 4

39yo female, Fatimah, an Iranian mother of 2 young children. Requires an interpreter. Supervision principles of stage 2 training. How to use feedback. Use of interpreters for the Iranian is woman. Prescribed antipsychotics and results in QTc interval of 450msecs, what would you tell the trainee to look for and how to manage this.

Answer as per Maudsley 25th edition

Question 5

You're a junior consultant hi just working in an adult community mental health clinic. You are referred Jason, a twenty-four-year-old nursing student, by his GP with concerned about few months off depressed mood, lack of motivation, energy. He's stressed because He has missed Time of University due to 3 four-day hospital admissions to the hospital so far this year already. Is now faced with an end of term

assessments/assignments and is stressed he might fail the year. He attends to see you and tells you that he has been feeling depressed in mood and stressed. He reports distress from arguments with his housemate who has been concerned about his picky eating, restriction of food and vomiting

5.1 What further information would you require for a targeted history and assessment of Jason? List and explain

Jason tells you during the assessment that during a recent nursing placement he was exposed to information about the patient's trauma/assault history. He confides in you and tells you that he himself was sexually abused at the age of 12 years old by his cousin and brother age 24 at the time. The recent exposure to events at placement has brought back memories.

5.2 How would you respond to Jason's disclosure at this stage? List and elaborate

You refer Jason to sexual assault counselling services, following which he returns after a week of counselling. He tells you that he is feeling much better, but still experiencing low moods and lack of motivation. He reports you still sometimes resorting to restriction of foods with self-loathing.

5.3 What psychological therapies would you consider for Jason at this stage? Lists and elaborate

Question 6

69yo married, retired high school teacher who lives with her husband. Forgetful. Forgetting her way around shopping centre, losing her way when driving, forgetting to turn off the stove. Sleep is disturbed. Usually had one alcoholic drink with dinner but lately been drinking more as her husband has been finding bottles of wine in the bin. Diagnosed with dementia. Husband wants her to remain at home. Discuss approaches to managing her (College has given the answers to this).

March 2020

MEQ 1. Root cause analysis - explain to trainee

ECT - give a lecture to trainees

Administration, procedure

ADHD prescribing - your prescribing varies to peers after results of survey published how do you approach

MEQ2

Child.

Purpose or advantage or use of play in assessment of children and adolescents

Use of projective tests in children

Trainee approaches you after lecture to say he is concerned about his child due to conflict with partner

MEQ 3

19yr old with acrophobia and panic

Give possible diagnosis

List psychosocial factors of concern

Give short term management

MEQ 4

54yr old Schizophrenia for 15yrs. Feels foggy and tired on Clozapine. Wants to change

What is your approach to request

How do you manage withdrawal of clozapine safely

Trainee asks about discontinuation and withdrawal symptoms of antipsychotics (7marks)

MEQ 5

Borderline patient 22yrs old. Taken overdose of paracetamol. Registrar assesses.

What are your IMMEDIATE treatment goals for patient

What are the disadvantages of continuing inpatient management 4marks

Conversation with Counsellor who is using retrieved memory therapy. What are ethical considerations

MEQ 1 (~30 marks)

Young adult male believes he has ADHD (community MH clinic setting). Not much more detail.

1.1 What is your approach to assessment?

1.2. What are the issues associated with prescribing in adult ADHD?

1.3 What are the non-pharmacological management strategies for adult ADHD? (8 marks)

MEQ 2

Extremely long stem >1 page long. You are a psychiatrist in community MH clinic. 70yo married ex vet referred by GP for longstanding ruminations about his wife's faithfulness, despite her reassurances to the contrary, depressed mood and sleep disturbance but denies self-harm ideation. Didn't respond to 2 different SSRIs so GP switched to moclobemide and quetiapine. Past history of sexual dysfunction secondary to antidepressants though this isn't currently an issue. Also, ruminations around a small payment from the department of vet affairs, left his employment because of this a number of years ago. Contains an excerpt from the GP referral letter. Then a long excerpt from your own letter back to GP after you assess the patient. The wife did admit to an indiscretion in the marriage 31 years ago around the time of the onset of the ruminations however maintains that it wasn't a big physical thing that the husband made it out to be, the husband's distress worsened after finding out about this. Wife denies any concerns about safety of herself or the family. Current meds are moclobemide 150mg BD, quetiapine 50mg nocte, sildenafil PRN. Gives some results of cognitive testing with the conclusion that the impairment may be related to poor effort due to distress during the examination. Your diagnosis is OCD (ruminations are somewhat relieved by wood chopping), with DDx of delusional disorder and depressive features but not meeting threshold for depression.

2.1 describe the phenomenology of your Dx and DDx (12 marks)

2.2. what are the risk issues in this patient? (8 marks)

2.3 debate your recommended medication plan for the GP (8 marks)

MEQ 3

You are a junior consultant in a city adult inpatient unit. Young adult male Nikolai admitted with psychosis, on 'an antipsychotic'. Not much more detail.

3.1 Explain how you interpret ECG in relation to psychiatry? (5 marks)

3.2 What is the relevance of QTc to psychiatry? (12 marks)

3.3 Nikolai's QTc is 470 after 1 week of medication, what is your management? (8 marks)

MEQ 4

You are a junior psychiatrist on a busy regional hospital inpatient unit. You supervise a registrar who is an international medical graduate who normally performs well. The NUM approaches you saying the registrar has been skipping lunch and staying 3 hrs late recently. He has also been irritable with staff and patients. After discussing with the NUM, you don't have any immediate concerns.

4.1 What is your approach to this?

4.2 The registrar admits to having difficulty with workload, on calls, studying for exams and missing his family. He asks you for advice on what to do. Describe your approach.

4.3 What are the barriers to the registrar implementing your advice?

MEQ 5 (18 marks)

Elderly woman living in a residential unit, confused and looking dysphoric lately, unsteady gait, on a TCA, L-dopa/carbidopa and atenolol. Gives detailed results of cognitive testing - MMSE 21/30, poor short-term recall, poor orientation, impaired on clock drawing, impaired visuospatial and frontal lobe impairment... can't recall all the cognitive test results.

5.1 What are the possible aetiologies for her presentation?

5.2 What are the principles of prescribing in the elderly?

5.3 Her son asks you about her making a will. What are the issues around testamentary capacity? (5 marks)

August 2021 Missing

Question 1:

The first MEQ presented an important opportunity to show content and the practical application of CBT. The question allowed candidates to think of a broad range of issues when conducting psychotherapy. Candidates in general were familiar with psychotherapies recommended in the treatment of depression. Marks were lost when candidates did not follow the specific instruction to debate the psychotherapy options. Candidates are reminded to make themselves aware of the instructions in each question. More information can be found in the guide 'MEQ Instructions to Markers',

Question 2:

MEQ 2 pertained to a common scenario of a woman with schizoaffective disorder who was considering pregnancy, her progress during the pregnancy, and subsequent to the birth of an infant. The questions pertained to issues around assessment, ethics, psychosis, and perinatal areas of the curriculum. Most candidates had a good grasp of key assessment components and understanding of key risk in the perinatal/post-partum period and paid due attention to the importance of family and social connectedness. There was also a good opportunity to explore how recovery principles apply to practice. Generally, in the third vignette, candidates appropriately identified the scenario as a psychiatric emergency and responded appropriately. Medico-legal aspects were generally considered. Examiners commented on the lack of comprehensive discussion about ethics, involvement of specialist teams, and broad discussion of the issues supporting the patient in the decision-making process. Answers often showed minimal understanding of concepts of recovery-based care. Furthermore, there was limited longer term planning or anticipation of care needs.

MEQ 3 Candidates performed relatively well on this MEQ, achieving on average 63% of available marks. MEQ 3 was very common scenario, which tests the width and breadth of management of delirium and alcohol withdrawal. The question covered a many aspects of the clinical care of a patient presenting with a substance abuse disorder including psychosocial issues, the welfare of a dependent as well as considering the differential diagnostic issues between mental health issues, medical or acute alcohol withdrawal, pain, and capacity. Many candidates did not arrange answers in a structured way and this meant that remembering to mention salient features of the history as applicable in this scenario – at times, the medical, psychological or drug and alcohol history, mental state examination, the physical examination or relevant investigations, was missed or incomplete. Markers commented on the candidates not providing the responses asked for, which led to markers not being awarded. Candidates are reminded they will not be awarded marks for not following the instruction in the question. In this MEQ, many candidates did not 'justify' their responses. For example, provision of a justification for asking about relevant aspects applicable to this scenario, or for ordering certain investigations. This MEQ was performed well with a 63% average marks.

MEQ 4 This vignette tested candidates' key understanding of the assessment of capacity and is essential in psychiatry. The vast majority of the cohort were able to identify that a non-delusional religious belief was often shared between other individuals of the same religious denomination. The majority of candidates lost marks for not sufficiently explaining their responses. Overall, the question about ethics and professionalism performed poorly, in line with previous performance in this area of practice. This MEQ was performed poorly with a 48% average marks.

MEQ 5 Generally, candidates performed poorly in this question with variable attention to basic life support, the emergency assessment of the obtunded patient, consideration of monitoring level of consciousness, and did not explore principles of justice beyond. Most candidates did not explicitly refer to the use of a sedation scale in their assessment of the unrousable patient. Some candidates failed to note that the patient was unrousable and focused their answer on management of aggression. Candidates had difficulty explaining why information was gathered. Stronger candidates explained how the information about psychiatric history and drug and alcohol history would be used to formulate hypothesised explanations for the presenting problem, choose appropriate management, and consider differential diagnoses, rather than just listing the components of a psychiatric and drug and alcohol history. Overall, most candidates did well in recognising the urgency of attending to physical assessment of a medically compromised patient in a psychiatric setting and requirement of immediate assessment and observations, seeking urgent medical input and considering transfer of the patient to a more appropriate setting. Most of the cohort recognised the need for least restrictive care and often involvement of family and patient in the process. There was a strong knowledge of non-pharmacological approaches to managing an acutely behaviourally disturbed patient. This MEQ was performed with a 51% average marks

Question 1

Consultant Psychiatrist for adult MH. An audit into the practices of inpatient unit has made recommendation to implement non-pharmacological strategies to decrease the use of PRNs for nighttime sedation.

- a. **List and outline how you would plan and implement the changes recommended in the audit. (10 marks)**
- b. **List and justify the recommendations for non-pharmacological strategies for insomnia you would be considering. (10marks)**
- c. **List and outline the normal physiology of sleep including the polysomnography. (8 marks)**
- d. **List and justify the salient features between nightmares and night terrors in children. (6 marks)**

MEQ 1

The first MEQ related to the important issue of sleep and its relevance to t psychiatry, and quality improvement. The question draws on a well-established knowledge base and requires some ability to consider how that knowledge can be used in a different context. Beyond a superficial understanding of quality and safety, there was little ability to demonstrate knowledge of change processes. Candidates often gave very generic advice and did not consider what strategies could be implemented at the level of the inpatient unit. Only a small number of candidates demonstrated an accurate understanding of the stages of sleep, and many had no understanding of the physiology of sleep. This is concerning given the extent of sleep disturbance in psychiatric patients

Question 2

Gary is a 39-year-old schoolteacher, that has had work performance issues and had a formal warning due to his performance over the past year. He initially presented with work difficulties, increasing irritability and bouts of anger which is not usual for him. He is also experiencing poor concentration, unsteady gait, dizziness and depressed mood and has self-referred through the GP for an assessment. He has also ended a relationship with his partner recently. He describes himself as a perfectionist. His mother was treated for bipolar disorder for 20 years and she passed away last year. Tests have come back to reveal that he is immunocompromised and that he recently found out he is HIV positive.

- a. **List and justify the differential diagnosis (8 marks)**
- b. **List and justify the psychological therapies that you would be considering for Gary (8 marks)**
- c. **List factors involved resulting the psychosocial consequences of HIV related illness. (6 marks)**

Gary has been stable for 18 months after receiving treatment with anti-retroviral and his HIV is stable. You elicit history of heavy cannabis use which Gary says has been keeping him stable.

- d. **List the consequences of cannabis use on Gary's treatment (4 marks)**

MEQ 2 is a common scenario of a man who is HIV positive and has substance use issues. The questions pertained to issues around epidemiology, substance use, psychological and treatments areas of the curriculum. There was a generally limited knowledge of psychotherapy modalities, and some candidates appeared to provide a scattergun list, often not pertinent to the scenario. Many candidates talked about grief and advocated for peer support and lived experience workforce as being instrumental in helping people on their recovery journeys. Most also considered the

impact of psychoactive substance on the patient's biological treatments. The majority of candidates demonstrated a good understanding of HIV and mental health. Of concern, some candidates made unfounded assumptions that the patient was homosexual and/or an adulterer. Some candidates appeared not to read the instruction carefully, providing detailed responses for "List" questions for which they were not awarded marks, meaning they lost valuable time in the exam

Question 3

Stage 3 trainee has been allocated to you and will be conducting a visit to a rural community of 400 with an indigenous mental health worker. The Indigenous mental health worker is related to one of the three families within the community of 400. The trainee reminds you that they are seeing a 23-year-old Indigenous male, Selwyn, that was recently released on a forensic order for 2 years after he was incarcerated for assaulting an elderly indigenous person due to untreated psychosis exacerbated by methamphetamine use. On admission his diagnosis was schizophrenia and was treated for this as such. However, on discharge, forensic mental health diagnosed Selwyn as having a moderate Intellectual Disability with specific deficits related to poor impulse control and verbal fluency. He has been stable and responded well on low dose antipsychotics.

- a. **List and explain the advantages and disadvantages of having Indigenous mental health worker in this scenario. (10 marks)**
- b. **List and justify the factors that would result in increased likelihood of Selwyn being incarcerated again? (10 marks)**

MEQ 3 tested knowledge of socio-cultural awareness and specific areas of practice – indigenous/Māori areas of the curriculum. The question asked candidates to list and justify factors associated with the likelihood of the patient being incarcerated in the future. Marks were lost when candidates did not follow the specific instruction to justify factors. Many candidates showed a good breadth of knowledge in indigenous mental health and forensic issues in rural settings.

question 4:

Sahiba Abdullah is a 42-year-old lady presented to outpatient clinic post overdose of amitriptyline 10mg x25, which she took as she believed her health problems would not resolve. She is remorseful, but continues to have chronically poor sleep, depressed mood, difficulty concentrating, muscular aches and pain which are increased on doing daily activities. Due to her health issues, she has been unable to do her duties at work as a tutor at the university. She has having conflict with her husband due to her inability to care for her 3-year-old as result of her symptoms. Her husband says that he does not believe that there is anything wrong with her and that he does not want her to be treated by a psychiatrist as their community would exclude them if they knew Sahiba was receiving treatment for a mental health condition.

- a. **List and explain the differential diagnosis likely in this scenario. (10 marks)**
- b. **List and explain the causes of stigma. (6 marks)**

They agreed to do psychotherapy, but then disengages and refuses to answer any calls.

c. What are the issues you would consider with the Registrar before discharge (8 marks)

This vignette tested several key aspects of clinical care and is a common clinical scenario. MEQ 4 assesses the basic essential skills in formulating differential diagnoses and identifies psychological cultural factors as management barriers. In general, candidates did well in providing a differential diagnosis and being able to elaborate and justify their offered disorders. Most of the cohort outlined several aspects of stigma without necessarily identifying them as such. Some candidates were familiar with systems and processes for managing a client who has disengaged from the community services. There was a general feeling of a greater level of sophistication evident in the responses provided in this MEQ. The majority of candidates lost marks for not sufficiently explaining their responses. Overall, the question about ethics and professionalism performed poorly, in line with previous performance in this area of practice. This MEQ performed strongly with a 65% average marks

Question 5

You're a consultant at a community clinic. Stan is a 37yo single, male, unemployed electrician living with his elderly parents who shop and cook for him. Can't leave the house due to with OCD, worries about electric sockets. He has had intermittent engagement with MH services but none lasting longer than 3 months. He has trialled many antidepressants but not tolerated any due to side effects.

5.1 Outline (list and justify) factors that may have hindered Stan's engagement with services in the past (6 marks)

You assess in his home and confirm he has no other major psychiatric disorder. Stan tells you that he has to check switches and power sockets constantly because he feels he will leave an electrical appliance on and call time to his parents. Similar fears have been preventing him from working outside of the home. He would like treatment for his symptoms as long as it doesn't involve admission to hospital.

5.2: Describe your comprehensive management plan for Stan (12 marks)

5.3 Outline pharmacological management (6 marks)

MEQ 5 was a question relating to OCD and tested a scenario often seen in the public sector. This MEQ covers assessment, treatments, and anxiety areas of the curriculum. Generally, candidates performed strongly in questions pertaining to pharmacological management strategies and had a good understanding of basic CBT principles. Weaknesses apparent include not being able to think more systemically when working with an adult patient, not addressing perpetuating factors (such as family factors), and a lack of knowledge of exposure-response prevention intervention. There was a tendency to not follow the instruction in the question (such as "list and explain"). This MEQ was performed with 57% average marks

MEQ 1 (27 marks)

You are a junior consultant in CL psychiatry in a large tertiary hospital. You are asked to see Janice, a 51-year-old homeless woman. She was admitted to the ED 36 hours ago. She was detained under the MHA by your colleague as she was presenting as psychotic and agitated. Janice was admitted to the mental health unit one month ago and absconded after 24 hours. In ED, she has been nursed by a 1:1 special. She has been aggressive and has spat on an RN. She has received two doses of midazolam and droperidol in the past 24 hours. The ED staff are concerned about her admission. She is still in ED awaiting a bed in the inpatient mental health unit.

1.1 Outline and justify what key factors/issues you would need to consider prior to reviewing Janice. (10 marks)

1.2 Outline and explain what advice you would give to reduce the risk of aggression in the ED (7)

1.3 ED consultant wants to discharge the patient.

1.4 Outline the ethical and governance issues pertinent to this case (10 marks)

The first MEQ related to a common ED presentation covering clinical and administrative realities that trainees would be familiar with. Curriculum areas of Assessment, ethics and Leadership, governance and legal frameworks were covered by this question. Most candidates were able to identify legal frameworks embedded in the question as important and were able to write from clinical experience. The MEQ touched on both ethical and clinical governance that were important concepts to draw out of this assessment. Whilst this MEQ performed well (60.7% average marks achieved), many candidates did not pay attention to the describe (list and explain) prompts, providing lists without expanding on their answers thus did not receive any marks.

MEQ 2 (39 marks)

You are a junior psychiatrist in a small rural town. You are caring for K, a 39 year old woman. She has a long history of **schizoaffective disorder** and is managed on clozapine **300mg** and lithium **750mg BD**. She is single and lives alone. She is financially supported on the DSP. K's symptoms are well controlled. Her last admission to the inpatient mental health **unit was three years ago**. She is obese with a **BMI of 35**. She does not use drugs or alcohol. K explains that she wants to have a baby and seeks your advice about pregnancy.

2.1 Outline and explain the key factors you would need to consider when responding to her request (12 marks)

K presents at 28 weeks pregnant. She is experiencing symptoms of relapse and her case manager is concerned. She is treated with a reduced dose of clozapine and lithium.

2.2 List and explain your immediate management of K. (10 marks)

2.2 Discuss the principles of recovery orientated practice that apply to this case (5 marks)

2.3 Discuss the general principles when advising patients treated with psychotropic medication regarding breastfeeding (12 marks)

MEQ 2 is a relevant question on pregnancy and highlights a serious and important clinical presentation which candidates need to be able to manage safely in clinical practice. The scenario provided a broad range of opportunities to demonstrate and explain the relevance of these principles in clinical practice. Principles of recovery-oriented practice reflecting core knowledge was a question where the average mark achieved was a disappointing 51%. As a cohort: there was some familiarity with the use of psychotropics in pregnancy, when a patient is breast feeding, and transmission across placenta or via breast milk. Candidates recognised the importance of a person's autonomy and ability to make life decisions. In this question, markers were pleased with the recognition of the importance of including a person's social network. Some candidates performed poorly because they listed recovery principles without explaining the relevance of these to this particular context. Few candidates demonstrated awareness of relevant frameworks for guiding recovery oriented mental health practice in the Australian and New Zealand contexts.

MEQ 3 (34 marks)

You are referred Tom, a 44-year-old married retired pharmacist. He has a 17-year history of somatic symptoms with no organic cause found. He is mildly frustrated by this. No past psychiatric history. On review, mild mood symptoms, mild anxiety symptoms. He recently returned from an international holiday and had a good time. Doing well at work. Father was a pharmacist too, who worked long hours. Mother has a diagnosis of chronic fatigue syndrome, and his brother has depression. Tom describes his marriage as being "perfect" like that of his parents. There is a GP letter outlining Tom is commenced on venlafaxine for low mood about 4 weeks ago at 75mg and increased to 150mg and has not had a good response Low mood (mild) Anxiety (mild) Somatic symptom; nausea, diarrhea, abdominal pain, low libido, headache He's had an abdo CT and a few colonoscopies which have been returned normal TSH, UEC, LFT, FBC negative

3.1 List the possible differentials (10 marks)

3.2 What Eriksonian stage is Tom at (4 marks)

You make a diagnosis of somatic symptom disorder.

3.3 Outline and discuss what information about psychological treatments you would give to Tom. (12 marks)

3.4 Outline and justify the information you would include in our letter to the GP advising him about Tom's ongoing management (10 marks)

MEQ 3 tested knowledge of somatoform disorders, professional communication and liaison and treatments. Markers felt this is a very good question in assessment a trainee's ability to think laterally in a clinical setting. The presentation of the patient within the question is something that is of reasonably high prevalence. Trainees

continued to demonstrate an unfamiliarity with current diagnostic nomenclature. Trainees continue to struggle with providing appropriate justification for their answers. A common recurring example evident was that trainees would list generalised anxiety disorder as a differential diagnosis, but their justification was limited to "appears anxious". This is insufficient to gain marks. Many trainees attributed the presentation to factitious disorder or malingering, often with very contrived reasoning. There was nothing in the question to have suggested this. Finally, trainees should be encouraged to read the questions. Often trainees justified the need for thyroid function tests as part of their organic differential diagnosis, but the question clearly stated that the thyroid function tests were normal already.

MEQ 4 was a repeat question relating to the relevance of ECG monitoring in psychiatric practice. This is regarded as relevant for day-to-day safe practice. This MEQ covers treatments, basic sciences, and medical knowledge areas of the curriculum. The vast majority of candidate performed poorly in this question. Most could not accurately state the relevant QTc intervals of concern for men and women. Many did not consider fatal outcomes; few elaborated on the risk factors for prolonged QTc other than pharmacotherapy. Many candidates appeared inexplicably to focus solely on the monitoring requirements for clozapine. Here again, marks were lost due to insufficient justifications or explanations.

MEQ 1 26 marks

You are a junior consultant working in the consultation-liaison service at a metropolitan hospital. Adil is a 32-year-old male Afghan refugee who has a previous history of schizophrenia and heroin misuse. He is managed by the local community mental health team and receives a depot antipsychotic. He is admitted to the medical ward with suspected infective endocarditis. He has been admitted for a couple of days and has had a couple days of IV antibiotic treatment. He suddenly refuses treatment and cardiac investigations and is requesting to leave the medical ward.

1.1 What are the factors which might be contributing to his treatment refusal? (5 marks)

He remains in hospital and agrees to ongoing treatment.?? Was he started on methadone here? The medical team contact you again and report he is appearing depressed and withdrawn.

1.2 what are your differentials

1.3 How would you manage this?

He is commenced on methadone 60mg and is referred to the local methadone program. You are seeing Adil for discharge planning meeting.

1.4 What is the information that you would discuss with him in regard to discharge planning?

The first MEQ related to a relevant presentation that trainees would be familiar with. Curriculum areas of assessment, management, sociocultural awareness, consultation liaison and treatments were covered by this question. Most candidates were able to identify legal frameworks embedded in the question as important and were able to write from clinical experience. The MEQ touched on ethical and clinical governance, which were important concepts to draw from this assessment. However, many candidates failed to address the question, instead focussing on general themes like risk assessment, capacity assessment, and exploring past psychiatric history rather than current issues. Many candidates also did not understand the importance of explicitly stating how and about what you would liaise with the treating team. Whilst this MEQ performed relatively well (57.75% average marks achieved), many candidates did not pay attention to the Outline (list and justify) prompts, providing lists without expanding on their answers and thus did not receive any marks.

MEQ 2 30 marks

You are a recently appointed junior consultant working for a small community mental health service. There are two part time consultants and one Stage 2 registrar. You previously worked as a registrar in the same team. The clinical director of the service has had to take urgent medical leave for 4 months, effective immediately. The senior executive manager requests that you step up into this role whilst the formal recruitment takes place.

2.1 What are important issues you need to consider? (15 marks)

You accepted the position. Two weeks into this new role, Sally, the stage 2 registrar, comes to you to complain about workplace sexual harassment and bullying from the male senior nurse. She says that she can substantiate these claims.

2.2 What are important issues you need to consider when managing this situation? (15 marks)

MEQ 2 is a relevant question on sexual harassment, bullying and leadership in the workplace which candidates need to be able to manage appropriately in clinical practice. Committee for Examinations (CFE) considered this question to be important to be assessed due to the issue being prevalent in society and the public reports identifying this issue as arising in clinical care settings. Please note that the scenario in the question was designed to prepare candidates for some of the particularly challenging predicaments they may find themselves in, even as a junior consultant. The College has an ongoing responsibility to care for, support and prepare trainees and SIMGs, from the commencement of and throughout training, to manage the range of challenging and potentially traumatic issues that may confront trainees, SIMGs, pre- and post-Fellowship, in clinical care and team settings. The College has an ongoing commitment to ensure that it has an awareness of these issues. Trauma informed practice statements are published by the College. Trauma-informed practice | RANZCP The College considers that engaging in discrimination, bullying and/or harassment is a professional and ethical issue, as reflected in the RANZCP's Code of Ethics and is never acceptable. The College is committed to providing a safe, inclusive, and welcoming environment for all members and aims to achieve this by having a zero-tolerance approach to any form of discrimination or harassment, including but not limited to sexual harassment. However, the College must continue its journey to ensure all members feel empowered to report sexual harassment. Many of the cohorts had careful consideration in their answers. Many trainees were able to identify the impact the role may have on personal wellbeing. They also explored the role itself in a thorough manner, acknowledging the importance of understanding roles, governance structures, and the impact of limited resources. However, many candidates appeared to feel that the leadership role operated without being connected to a larger organisation and to systems of support and did not discuss consulting about decisions. The question provided an interesting insight into the perception of leadership and leadership roles. Candidates who could maintain a systematic approach were able to achieve more marks.

MEQ 3

You are working as a junior consultant for a community mental health team. You are referred Neville, aged 60 years, who works part-time in a boat-building yard laminating and painting boats. He is a Returned Serviceman/Army Forces Veteran and saw active service overseas. He is divorced, lives alone, and is a heavy smoker. A few nights ago, he was brought into the emergency department by his girlfriend of 2 months because he has injured his leg trying to jump out of bed in the middle of the night. Neville's girlfriend says that sometimes when she stays the night, Neville wakes sweating and screaming, and if she touches him during the episode, he can be aggressive. She says that he doesn't know what he is doing during the episodes. She says that this evening's episode was worse than usual. Today, Neville smells strongly of alcohol, and says he "can't remember" what happened this evening. You ask him about war experiences and the effect of those experiences on his mental health. Neville is a reluctant historian but says that he served as a soldier in the Vietnam War. He says that he doesn't like to think about the past but, when pressed, he says that lately he has experienced "visions" of events during the war.

3.1 What difficulties might arise in the interview process? (9 marks)

Neville eventually seems to relate quite well to you, saying that he has "never told any of this stuff to anyone before". He wants to know more about treatment.

3.2 Discuss your comprehensive assessment (?10 marks)

3.3 Discuss non-pharmacological management of Neville (?10 mark)

MEQ 3 tested knowledge of trauma-related scenario that is complicated by substance use and relationship stressors and management of this situation. This MEQ had an average of 44% marks achieved. There were some significant deficits in terms of knowledge. Candidates tended to adopt an approach to understanding problems in engagement that ignored how the context of the meeting might affect engagement - so that the role of triaging and assessment, the role of treatment setting, the role of legal structures (related, for example, to coercive treatment or privacy and confidentiality) and the relevance of the consumer's personal relationships were overlooked; instead candidates tended to focus on more pathologizing views of the consumer. Similarly, there was less consideration of the clinician's potential to stigmatise the consumer or consideration of how factors such as gender, sexuality and ethnicity might influence the way the power dynamic between doctor and patient is played out. An important weakness was the tendency to overlook physical health in this patient's presentation. Few candidates articulated that a review or examination of the patient's physical state is important even though there were potential physical health comorbidities. In terms of offering care to this consumer, candidates tended to focus on treating the diagnosed condition (PTSD) without considering how evidence for treatment of this disorder could be incorporated into a person- centred care plan for this consumer. As such, the role of addressing alcohol use was less considered; and the way that care could be recovery oriented was much less clear. There was often poor consideration of social and vocational parameters of recovery or the dilemmas with how the partner could be included in the consumer's care planning. Markers discourage the use of abbreviations unless they are extremely common. This is particularly the case for blood tests which often change over time and between jurisdictions. Poor exam technique included not justifying (just listing) or inadequate or irrelevant justification.

MEQ 4 25 marks

Lisa, aged 14 years, lives with her parents and younger sister. She is an excellent student in Year 10 and has until recently been very involved in competitive gymnastics. Lisa has been in a Paediatric ward for a week with a history of intermittent nausea and weight loss over several months. She has had a full medical assessment including an abdominal ultrasound, a gastroscopy and various blood and faecal tests. The paediatric gastroenterologist does not consider her symptoms to be due to any obvious pathological process and plans to discharge her today with a six-week follow-up appointment. The nursing staff ask for referral to psychiatry because they have noted that she avoids food, and she has lost further weight while in hospital. They are concerned that she has an eating disorder.

4.1 Describe how you would assess Lisa's eating behaviour (8 marks)

Following your assessment, you make a diagnosis of anorexia nervosa restrictive sub-type. There is no other co-morbid psychiatric diagnosis. Lisa has a Body Mass Index (BMI) of 14.5. You discuss the results of your assessment with Lisa and her parents and start arranging ongoing management. Several hours later the paediatric team contacts you to say that Lisa must be transferred to a psychiatric ward because Lisa's bed is needed for medical patients.

4.2 What would you do in this situation? (8 marks)

There are no beds available in the psychiatric unit and the specialist team that manages young people with eating disorder cannot see her for six weeks. Lisa needs ongoing inpatient care and the paediatric service agrees to Lisa remaining on a medical ward.

4.3 Describe how you would manage Lisa in the paediatric ward (9 marks)

MEQ 4 covered broad areas of psychiatric patient care in a consultation liaison setting with the importance of liaising with all pertinent caretakers and systems along with cultural and social issues, including specifics for drug dependency management, medical issues, and trauma related issue. It also focuses on details of migrant care management. This MEQ covers eating disorders, basic sciences, medical knowledge, professional communication and liaison and treatment areas of the curriculum. Most candidates performed poorly in this question; many candidates failed to recognise that this was a consultation liaison question and an eating disorder question, resulting in generally low marks. This MEQ performed poorly, with 44% average marks achieved.

MEQ 5 14 marks

You are a consultant working in a community mental health team. You are treating a young woman on combination of mirtazapine and venlafaxine for treatment resistant depression. She has recently had a relationship breakup. She has tried 3+ antidepressants since age 14. She has previously tried CBT and attended 2-3 sessions. She didn't like the psychologist as she gave her homework to do.

5.1 What is your approach to assessment of someone with treatment resistant depression? (5 marks)

5.2 outline reasons for treatment resistant depression

She has gained 15kg in weight and feels that her partner broke up with her because she was fat. She tends to indulge in comfort food - eating ice cream and sweet food at night.

She later represents to you stating she was close to overdosing on paracetamol and has inflicted superficial lacerations to her abdomen. She denies feeling suicidal and assures you of her safety.

5.3 What is your comprehensive management of her at this time? (9)

2023 - August

MEQ 1

The first MEQ was a governance question about the **use of restrictive interventions in an inpatient unit**. Some, candidates were able to think broadly about factors that impact on restrictive practice. Overall, the domains relating to staff and systemic factors impacting on restrictive treatments were well covered. Domains relating to patient factors contributing to restrictive practices were less well answered, such as male gender, psychotic illness' 1 performed poorly (40% average marks achieved), many candidates did not pay attention to the Describe (list and explain) prompts, providing lists without expanding on their answers and thus did not receive any marks. Whilst in the "List" question, candidates expanded on answers which was unnecessary.

Similar theme questions:

MEQ 2

Asks the candidate to discuss a proposition that is relevant to clinical practice. The questions required candidates to consider policy, research and professional/ethical aspects associated with a highly relevant clinical dilemma. Candidates tended to understand the **workplace-related challenges** associated with inclusion of physical health care, to expound on the relevant overlap between physical and mental health and to cite specific examples of physical health concerns that manifest psychiatric symptoms. Some candidates were able to identify the role of supervision in addressing knowledge and practice gaps for the trainee mentioned in the vignette. Many candidates had little knowledge of the college standards on physical health and the need for ongoing professional development. Candidates were aware of physical health concerns associated with a person who had a possible first episode and who had been restrained. Answers acknowledged some degree of problem with the term "**medically cleared**" although this was not often articulated well. Question covered curriculum areas of Leadership, Governance and relevant legal frameworks, Professional communication, Basic sciences, medical knowledge and Advocacy.

Theme: CLP question request to transfer the patient and issues

MEQ 3

Tested knowledge of assessment of addiction and anxiety and management of these. This MEQ performed well with an average of 67% marks achieved. Generally, a holistic, collaborative and patient centered approach was adopted by a majority of candidates. They demonstrated a good understanding of psychological and lifestyle management factors for anxiety disorders including panic disorder. Few candidates addressed the clinical presentation completely rather splitting the answer into two separate clinical problems. **This was an addiction/dual diagnosis question involving a patient who developed dependence on alprazolam for panic disorder. (Repeat 2025)**

MEQ 4 Indigenous Case

MEQ 5 covered treatments, basic sciences and medical knowledge areas of the curriculum based around an ECT scenario. Most candidates identified the important aspects of administration of ECT safety. The cohort were able to clearly detail that consent was required, the legal process for patients unable to consent and that they would use multimodal approaches to provide teaching to trainee registrars. This MEQ had an average mark of 49% achieved, which was disappointing given that most trainees would complete their ECT EPA in Stage 1.

MEQ 1

(Adil, aged 32)

The first MEQ, about a young refugee male in a general medical ward. This was an opportunity provided to adopt a holistic approach to his care, with cultural and social considerations. It involved many facets of a patient in hospital with several diagnoses. It was a practical question about a scenario that could easily come up as a junior consultant working in a consultation-liaison role.

Candidates missed marks for inadequate justifications to their responses provided, or for not considering liaising

with the medical team, for example. Systems issues were inadequately addressed.

This MEQ tested the candidate's ability in key competencies areas of assessment, sociocultural awareness, professional communication and liaison and treatments. This MEQ can be found on the College website.

MEQ 2

(Daniel, aged 12) Mood disorder differentials in childhood

MEQ 2 was about an indigenous child in out-of-home care and who presents to the emergency department. This was considered a good question in that this is very typical clinical situation that many psychiatrists encounter at times. It emphasizes not just the biopsychosocial approach to care but stresses the importance of intergenerational, cultural, context-dependent, and moral/spiritual factors in the care we provide.

There was a strong focus on the potential stigma of having a diagnosis of a major mental illness from a young age.

Many candidates did well in the first question regarding the possible reasons for Daniel's presentation. Candidates also did well to individualize Daniel's behavior management plan, taking his developmental needs into account. Candidates rarely considered seeking information from a variety of sources to contextualize the diagnosis that was made. A reasonable proportion of candidates did not expand on answers and thus missed out on marks. Many did not consider nor mention cultural factors, racial discrimination, and negative staff attitudes as reasons for this child's situation.

MEQ 2 covered curriculum areas of assessment, epidemiology, diagnosis and classification and public health, treatments in psychiatry and treatments.

MEQ 3 (Nicolette) – ECG Maudsley – Direct question

MEQ 3 was a Basic Sciences question and provided an opportunity to express a more advanced clinical understanding of interpretation of an ECG. The question was highly applicable to clinical work involving the prescription of antipsychotic medication. Most candidates showed a broad understanding of elements of the ECG trace and QTc prolongation.

We continue to see lists without justification.

This MEQ has previously been used and is available on the College website. MEQ 3.2 – 3.5 has been extensively rewritten based on marker feedback. This MEQ performed reasonably well with 60.87% average marks achieved.

The question tested knowledge of basic sciences, medical knowledge, and treatments in psychiatry.

MEQ 4 (Julia, aged 52)

MEQ 4 was about a 52-year-old woman who had self-ceased her SSRI medication and covered assessment for discontinuation/withdrawal and differentiating it from a relapse of her anxiety disorder. This MEQ did not perform well with an average of 40% marks achieved. Generally, the cohort had some recognition that relapse and discontinuation/withdrawal symptoms can be difficult to distinguish due to similarity of symptoms. Candidates mostly appreciated the basics of how to manage withdrawal. However, a large number of candidates appeared not to have read the question properly. Candidates often listed factors that are relevant but failed to justify them - listing important factors or symptoms clusters does not tell the examiner whether this feature is suggestive of withdrawal or relapse - the differentiation is key to justifying why any factor is considered important. As a specific example, candidates often stated that the timing of onset of symptoms was important. This is true, but candidates often failed to justify this by stating whether a particular timing pointed more towards relapse or withdrawal.

MEQ 5 (Sleep)

This question about an audit should be familiar to candidates. The question tests understanding of process rather than seek a regurgitation of content. The second part of this question is considered a straightforward knowledge test and is relevant, both in terms of assessing issues relating to sleep in a ward.

There was some familiarity with the context of an inpatient acute unit and how it runs and the relevance of completing an audit in relation to sleep. Candidates seemed to have an appreciation of some basic sleep hygiene approaches that they could consider in the context of the inpatient unit. They seemed to appreciate that there were individual and environmental/contextual factors that would affect sleep and suggested reasonable options for mitigating those factors.

Candidates generally seemed to have limited experience of completing audits. Some candidates appeared to be “doctor-centric” and failed to consider the role of nursing, and importantly, allied health and the lived experience workforce in the conduct of this audit. Despite this being a repeated question, the average percentage of marks was 43.6%

MEQ 5 covered Leadership, governance and relevant legal frameworks, professional communications and liaison assessment, Specific disorder – sleep and treatment areas of the curriculum.

MEQ March 2025

1. 32yo pregnant lady, diagnosed with schizoaffective disorder for 12 years. She is on Valproate 1500mg, Lithium 900mg, Quetiapine 300mg. Had multiple admissions and relapses in the past. Difficult to manage, stabilized on the current combination. Voluntary consumer managed by community mental health team. Admitted to IPU voluntarily. (28M)

- 1.1 What information you would like to collect before assessing her. (10Marks)

She is 12 weeks pregnant. CPS involved with 3 other children. Currently euthymic and stable mental state. Community team wants to cease Valproate. She doesn't want to repeat past relapses.

- 1.2. What are ethical concerns? (9marks)

She had USG, it shows possible anomalies. She asked for USG report.

- 1.3. What is your approach? (9marks).

2. 52 yo female admitted to ED for 36 hours, 1:1 nursing special, aggressive, disrobing. Diagnosed with schizoaffective disorder, Methamphetamine use. Psychiatrist put her on MHA, she is waiting for bed in inpatient unit. She received droperidol and Midzolam for agitation but not settled. She spat on nurse needed screening for blood born virus screen. ED staff is unhappy to continue her in the ED. (28 marks)

- 2.1. What are clinical and system issues? (10marks)

- 2.2 How to manage her aggression? (10marks)

ED consultant says discharge her as she is almost 2 days in the ED.

- 2.3. What are ethical and governance concerns? (8marks).

3. 17yo Sandy admitted to ED with wrist cut. Currently low risk. It happened after comments on social media. Sandy prefers They or them pronoun. Sandy was good in academic in year 11. There is decline in performance for few months. They have concerns related to gender. (25marks)

- 3.1. What is your assessment (12marks)

They don't want to use their pronounce. As they heard nurse laughing outside the cubicle. They don't want return to ED again and wants to leave.

3.2. What is the source of stigma? (5marks)

3.3. What is your approach to Sandy's concern? (8marks)

4. Philip is a 27yo married male on Paliperidone depot for methamphetamine use and drug induced psychosis. He had multiple admission and managed on CTO. CTO revoked. He comes to you and ask for cease the depot. He is willing to work with you. (28marks)

4.1. What are the factors to consider for his request? (10marks)

He mentioned he developed sexual dysfunction and marital issues due to Paliperidone. He is willing to continue depot for now for few weeks.

4.2. What is your assessment? (9marks)

His Prolactin level 1500ng/L, normal is 15ng/L.

4.3 What is your management? (9marks)

5. 30 yo father, working in a IT company. Diagnosed as anxiety by a psychiatrist and prescribed Alprazolam 12 years ago. He continued it from GP till now. GP doesn't want to prescribe as the amount of use of Alprazolam. He denied other substance use, alcohol use. Continuous to have symptoms. (21marks)

5.1. What is your assessment? (11marks)

You diagnosed him with panic disorder and benzodiazepine dependence.

5.2. How do you manage him? (10marks)